

Media Information

From: Main Street Radiology

To: [REDACTED]

Page: 1/2

Date: 11/12/2025 5:54:11 PM



Bayside 32nd Ave	[REDACTED] Francis Lewis Blvd, Bayside, NY 11358
Bayside 44th Ave	44-01 Francis Lewis Blvd, [REDACTED] NY 11361
Downtown Flushing	136-25 37th Ave, Flushing, NY 11354
Western Queens	72-06 Northern Blvd, Jackson Hts, NY 11372
Glendale	85-14 Cooper Ave, Glendale, NY 11385

WONG LAP KOON, MD
[REDACTED] SANFORD AVE, SUITE 2A
FLUSHING, NY [REDACTED]

Referrer Phone: [REDACTED]
Name: [REDACTED]
Phone: [REDACTED]
Gender: Female
DOB: [REDACTED]
Patient: [REDACTED]
Exam Date: 10/15/2025 12:13 PM

Acc #: 10167656

Secondary ID: [REDACTED]

Exam Name: [REDACTED]

EXAM: MA 3D DIGITAL DIAGNOSTIC MAMMOGRAPHY (LEFT BREAST), US DIAGNOSTIC BREAST LEFT

History: 71 years old female presents for evaluation of left breast mass seen on recent screening mammogram. Patient [REDACTED]

Views obtained: LEFT 3-D tomosynthesis MLO/CC spot compression and ML views.
Digital mammography was performed with additional image analysis utilizing CAD.

Comparison: 9/23/2025, 1/29/2020, [REDACTED]

FINDINGS:

There are scattered areas of fibroglandular density.

Left upper central breast, posterior depth there is a persistent irregular mass measuring 0.7 cm.

BILATERAL BREAST ULTRASOUND, performed around the clock:

Comparison: 1/29/2020, 6/12/2018.
Breast composition: Heterogeneous

RIGHT BREAST:

No [REDACTED]
6:00, 1 cm from nipple there is a shadowing calcified nodule measuring 0.4 cm, stable.

LEFT BREAST:

10:00, 2 cm from nipple there is an ill-defined mass measuring 0.4 x 0.3 x 0.4 cm, likely corresponding to the mammographic mass.

No [REDACTED] axillary lymphadenopathy.

This fax was sent with GFI FaxMaker fax server. For more information, visit: <http://www.gfi.com>

Continued...

Main Street Radiology

Name: [REDACTED] J [REDACTED]

Date of Birth: [REDACTED]

IMPRESSION:

Left 10:00 sonographic 0.4 cm ill-defined mass, likely corresponding to the left upper central breast mammographic persistent irregular mass. Left breast ultrasound-guided core needle biopsy with clip placement and postprocedural mammogram is recommended.

RECOMMENDATION:

- 1. Recommendation: Ultrasound Biopsy. Left breast ultrasound-guided core needle biopsy with clip placement and postprocedural mammogram.*
- 2. If the clip does not correlate with the mammographic mass, stereotactic core needle biopsy would be recommended.*

BI-RADS CATEGORY: 4 - Suspicious Abnormality

Message left with medical secretary at Dr. WONG LAP KOON MD's office on [REDACTED] 12:20 PM. A copy of this report will be faxed to the doctor's office.

A letter has been sent to the patient [REDACTED]

Approximately 10% of breast carcinomas are not radiographically detectable. A negative report should not delay biopsy if a clinically suspicious mass is present. Dense breasts may obscure an underlying neoplasm.

BIRADS 0: Incomplete - Need Additional Imaging Evaluation and/or Prior Mammograms for Comparison

BIRADS 1: Negative

BIRADS 2: Benign

BIRADS 3: Probably Benign

BIRADS 4: Suspicious Abnormality - Biopsy should be considered.

BIRADS 5: Highly Suspicious of Malignancy

BIRADS 6: Known Malignancy

The New York State Department of Health requires us to indicate the date of the patient's last clinical breast examination.

Electronically signed by: Mrugank Shukla MD [REDACTED] 12:21 PM

Workstation: OUTREAD11

Report Electronically Signed by: Mrugank Shukla MD

Report Electronically Signed on: [REDACTED] 12:21 PM

This fax was sent with GFI FaxMaker fax server. For more information, visit: <http://www.gfi.com>

Document Information**Other: Outside Correspondence**

mmg and US

██████████ 00:00

Attached To:
Karen Shih

Source Information

Kayshla Maldonado | Baic Bre Srg
